

Medroxyprogesterone acetate (Provera®)

Medroxyprogesterone acetate is a cancer drug. It is used to treat breast, kidney and womb cancer.

On this page

- [What is medroxyprogesterone acetate \(Provera®\)?](#)
- [How medroxyprogesterone works](#)
- [How medroxyprogesterone is given](#)
- [About side effects](#)
- [Common side effects](#)
- [Other side effects](#)
- [Other important information](#)
- [How we can help](#)

What is medroxyprogesterone acetate (Provera®)?

Medroxyprogesterone acetate is a hormonal therapy drug. It is also called Provera®. It is used to treat:

- breast cancer (after menopause)
- kidney (renal cell) cancer
- womb (endometrial) cancers.

Medroxyprogesterone might also be used to treat symptoms caused by any cancer type, such as:

- poor appetite, causing you to lose weight
- hot flushes caused by some prostate cancer treatments.

It is best to read this information with our general information about [hormonal therapies](#) and the [type of cancer](#) you have.

Your cancer team will talk to you about this treatment and its possible side effects before you agree ([consent](#)) to have treatment. They will explain why you are having medroxyprogesterone and how long you will take it for.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the [electronic Medicines Compendium \(eMC\)](#) website, which has patient information leaflets (PIL) for individual drugs.

How medroxyprogesterone works

Hormones are chemicals that our bodies make. They act as messengers and help control how cells and organs work. Hormonal therapy drugs change how hormones are made or work in the body.

Many cancers need hormones to grow. Medroxyprogesterone is a drug that is similar to the hormone progesterone. Doctors think it may work by interfering with the hormone balance in the body. This may stop the cancer growing.

How medroxyprogesterone is given

You have medroxyprogesterone as tablets you can take at home.

During your treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

Taking medroxyprogesterone tablets

Your cancer team will discuss your treatment plan with you.

Always take the tablets exactly as they tell you to. This is important to make sure they work as well as possible for you. Make sure you:

- swallow them whole with a glass of water
- take them at the same time every day.

If you forget to take the tablets, take the missed dose as soon as possible within the same day. If a full day has passed, do not take a double dose.

Other things to remember about your tablets:

- Keep them in the original package and at room temperature, away from moisture, heat and direct sunlight.
- Keep them safe, where children cannot see or reach them.
- If you are sick (vomit) just after taking them, do not take an extra dose. Take your next dose at the usual time.
- Get a new prescription before you run out, and make sure you have plenty for holidays.

- Do not throw away unused tablets. Return them to a pharmacy.

Your cancer team may also give you other medicines to take home. Take all your medicines exactly as they tell you to. Do not stop taking any of your medicines unless your doctor tells you to.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Other cancer treatments may cause different side effects. If you are also having other cancer treatment, you may have other side effects.

When you have this treatment, you will have regular appointments with a doctor, nurse or pharmacist. Always tell them about any side effects you have. They can give you:

- drugs to help control most side effects
- advice about managing side effects.

Most side effects can be managed. But sometimes side effects are harder to control. It is important not to stop taking hormonal therapy without telling your doctor. If side effects cannot be managed, your doctor may suggest a different type of hormonal therapy.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Common side effects

These side effects happen to 1 to 10 people in every 100 people (1 to 10%) who have this treatment.

Increased appetite and weight gain

This treatment can make you feel more hungry than usual. This can cause you to [gain weight](#). This effect on your appetite will go away when you stop taking the drug. If you are worried about gaining weight, talk to your doctor or nurse.

Because of this side effect, this treatment is sometimes used to treat people who have lost interest in food and are [losing weight](#).

Feeling sick

Some people [feel sick](#), especially during the first few weeks of taking this treatment. Tell your doctor if you feel sick. They can prescribe anti-sickness drugs to help.

Constipation

This treatment can cause [constipation](#). Constipation means that you are not able to pass stools (poo) as often as you normally do. It can become difficult or painful. Here are some tips that may help:

- Drink at least 2 litres (3½ pints) of fluids each day.
- [Eat high-fibre foods](#), such as fruit, vegetables and wholemeal bread.
- Do regular gentle exercise, like going for short walks.

If you have constipation, contact the hospital on the 24-hour number for advice. They can give you drugs called laxatives to help.

If you have not been able to pass stools for over 2 days and are being sick, contact the 24-hour number straight away.

Fluid build-up

This treatment can cause a build-up of fluid in the body. This will slowly get better after treatment ends. Contact the hospital on the 24-hour number if you:

- are gaining weight
- have swelling in your face, legs or ankles.

They can give you advice and treatment to help.

Headaches

This treatment may cause headaches. If you have headaches, tell your doctor, nurse or pharmacist. They can give you advice about painkillers that may help. Contact the hospital straight away on the 24-hour number if you have a migraine or severe headache.

Feeling tired (fatigue)

[Feeling tired](#) is a common side effect. Try to pace yourself and plan your day so you have time to rest between activities.

Being physically active can help to manage tiredness and give you more energy. It also:

- helps you sleep better
- reduces stress
- improves your bone health.

If you feel sleepy, do not drive or operate machinery.

Difficulty sleeping (insomnia)

Sometimes this treatment can affect your sleep. Tell your doctor, nurse or pharmacist if cancer treatment makes it [difficult to sleep](#).

Dizziness

This treatment can cause dizziness. If you have dizziness, do not drive or operate machinery. Tell your doctor or nurse if you notice this side effect.

Sexual effects

It is common to lose your [sex drive](#) and have erection difficulties with hormonal therapy. Hormonal therapy may also make your penis shorter and your testicles smaller.

Your doctor can prescribe drugs and treatments to help with erection difficulties. These will not increase your sex drive. But they might work, even with a low sex drive.

Things can improve a few months after you stop taking the drug. But it depends on how long you take hormonal therapy, and what other treatments you have had. Some people continue to have problems after treatment ends.

Increased sweating

This treatment can cause increased sweating in some people. Wearing natural fabrics, such as cotton, may be more comfortable if you are affected.

Other side effects

These side effects happen to less than 1 in 100 people (less than 1%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Mood changes

You may feel low or depressed, or have mood swings, during this treatment. Talking to family and friends about how you feel might help. If mood changes last for more than a few weeks, tell your doctor, nurse or pharmacist. They can talk to you about different ways to manage low mood or [depression](#).

Blood clot risk

Cancer and some cancer treatments can increase the risk of a [blood clot](#). Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing.

Always call **999** if you have:

- chest pain
- difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Skin rashes

You may get a skin rash that looks like acne.

Always tell your doctor or nurse about any [skin changes](#). They can give you advice and may prescribe creams or medicines to help. Any changes to your skin are usually temporary and improve when treatment finishes.

Hair thinning

While you are having this treatment, your hair may become thinner, drier and more brittle. This is usually mild. If you are worried about this, ask your nurse for advice on hair care.

Raised blood sugar levels

This treatment can raise your blood sugar levels and increase the risk of diabetes.

Signs of raised blood sugar include:

- feeling thirsty
- needing to pass urine (pee) more often than usual
- feeling more tired than usual.

Tell your doctor or nurse if you have these symptoms. They can do tests to check your blood sugar level and give you advice.

If you already have [diabetes](#), your blood sugars may be higher than usual. You may need to check them more often, or make changes to your diabetic treatment. Your diabetic doctor or nurse will talk to you about how to manage this.

Breast changes

Your breasts may feel sore or painful. This is not harmful, but let your doctor know if it happens. They can prescribe painkillers if needed.

Changes to periods

Your periods will usually stop while you are having this treatment. Sometimes, you may have light vaginal bleeding (spotting). If this happens, let your doctor know. When you stop taking medroxyprogesterone, you may have some bleeding from the vagina, similar to a period.

Diarrhoea

This treatment may cause [diarrhoea](#). Diarrhoea means passing more stools (poo) than is normal for you, or having watery or loose stools. You may also have stomach cramps. If you have a stoma, it may be more active than usual.

If you are passing loose stools 3 or more times a day and this is not normal for you, contact the hospital as soon as possible on the 24-hour number. Follow the advice they give you about:

- taking anti-diarrhoea medicines
- drinking enough fluids to keep you hydrated and to replace lost salts and minerals
- any changes to your diet that might help.

They might also ask you for a specimen of your stool to check for infection.

Effects on the liver

This treatment may affect how your liver works. This is usually mild and goes back to normal after treatment ends. You will have blood tests to check how well your liver is working.

Allergic reaction

This treatment might cause an allergic reaction. But this is not common. Signs of a reaction can include:

- a skin rash or itching
- feeling breathless or wheezy
- swelling of your face, mouth or throat.

If you feel unwell or have any of these signs, contact the hospital straight away on the 24-hour number. Do not take any more of this treatment until you have checked with them.

Always call 999 if swelling happens suddenly or you are struggling to breathe.

Effects on the heart

This treatment can affect how the heart works. You may have [tests](#) to check how well your heart is working. These may be done before, during and after treatment.

If the treatment is causing [heart problems](#), your doctor may change the type of treatment you are having.

Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- breathlessness
- dizziness
- changes to your heartbeat (palpitations)
- swollen feet and ankles.

Other conditions can cause these symptoms, but it is important to get them checked by a doctor.

Always call **999** if you have:

- chest pain, pressure, heaviness, tightness or squeezing across the chest
- difficulty breathing.

Eyesight changes

Rarely, this treatment can affect your eyesight. Contact the hospital straight away on the 24-hour number if you have any changes in eyesight or loss of vision.

Second cancer

This treatment can increase the risk of developing a second cancer called a [meningioma](#). This is rare. The benefits of treatment usually far outweigh this risk. Your doctor, nurse or pharmacist can give you more information.

Other important information

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- [herbal drugs and complementary or homeopathic therapies](#)
- recreational drugs – for example, cannabis.

Contraception

Your doctor, nurse or pharmacist will advise you not to [get pregnant](#) or [make someone pregnant](#) while having this treatment and for some time afterwards. The drugs may harm a developing baby. It is important to use contraception to prevent pregnancy. Follow their advice about:

- what types of contraception to use
- how long after treatment you should continue to use contraception.

Breastfeeding

You are advised not to [breastfeed](#) while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Visit the [electronic Medicines Compendium \(eMC\)](#) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

[Cancer Information Development team](#)

Author

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
-

use illustrations to explain text

- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

[Get help](#)